

SENATE No. 849

The Commonwealth of Massachusetts

PRESENTED BY:

Julian Cyr

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act to ensure health care as a right.

PETITION OF:

NAME:

Julian Cyr

DISTRICT/ADDRESS:

Cape and Islands

SENATE No. 849

By Mr. Cyr, a petition (accompanied by bill, Senate, No. 849) of Julian Cyr for legislation to ensure health care for all. Health Care Financing.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act to ensure health care as a right.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 (a) Notwithstanding any general or special law to the contrary, the following terms shall
2 have the following meanings unless the context clearly requires otherwise:

3 “Single payer benchmark”, the estimated total costs of providing health care to all
4 residents of the commonwealth under a single payer health care system in a given year.

5 “Single payer health care”, a system that provides publicly financed, universal access to
6 health care for the population through a unified public health care plan.

7 (b) The center for health information and analysis shall recommend a methodology to
8 develop a single payer benchmark. The single payer health care system considered under the
9 single payer benchmark shall offer continuous, comprehensive and affordable coverage for all
10 residents of the commonwealth regardless of income, assets, health status or availability of other
11 health coverage. The benchmark may consider the costs of a single-payer health care system at
12 different actuarial values, levels of cost-sharing and levels of provider reimbursement; provided

13 however that the benchmark shall include all actuarial values, levels of cost-sharing and levels of
14 provider reimbursement considered by the center. In developing the methodology, the center
15 shall monitor, review and evaluate reports related to single payer health care and the
16 performance of single payer health care systems in other states and countries.

17 (c) The center for health information and analysis, in conjunction with the health policy
18 commission and the division of insurance, shall provide an annual report detailing a comparison
19 of the actual health care expenditures in the commonwealth for 2025, 2026 and 2027 with the
20 single payer benchmark for 2025, 2026 and 2027, respectively, indicating whether the
21 commonwealth would have saved money while expanding access to care under a single payer
22 health care system. The first report shall be filed by not later than July 1, 2027 to the clerks of the
23 senate and house of representatives, the joint committee on health care financing and the senate
24 and house committees on ways and means.

25 (d) If a report under subsection (c) determines that the single payer benchmark
26 outperformed the actual total health care expenditures in the commonwealth in 2025, 2026 and
27 2027 the health policy commission shall submit a proposed single payer health care
28 implementation plan to the clerks of the senate and house of representatives, the joint committee
29 on health care financing and the senate and house committees on ways and means within 1 year
30 of the date that the report is filed. The plan may include proposed legislation to implement a
31 single payer health care system that offers continuous, comprehensive and affordable coverage
32 for all residents regardless of income, assets, health status or availability of other health
33 coverage. When developing the implementation plan, the commission shall hold not less than 3
34 public hearings and seek stakeholder input from across the commonwealth.”.